Design Brief

Problem Statement

In the Rohingya refugee camps, critical challenges impact menstrual hygiene management, including limited clean water, inadequate sanitation facilities, and restrictive cultural and religious beliefs that affect women's rights. Only 4.35% of women and girls receive adequate education on these matters further compounded by conservative opposition that limits their access to necessary health resources [1]. According to studies [2], reproductive tract infections are around 70% more common among woman who had poor menstrual hygiene practices. Thus, addressing this problem is important to prevent the spread of diseases in the camps by a large margin. This requires improved educational opportunities for women, enhanced menstruation facilities, and increased awareness among men to support women's health needs.

Key Stakeholders

Table 1 summarizing stakeholder examples, their needs and level of influence

GROUP	STAKEHOLDER	NEED	INFLUENCE
Direct-End Users	Women and girls	Require clean water, private and safe sanitation facilities, sanitation product, menstruation education	LOW
Pad Manufacturer	Freedom (Bangladesh-Based) [3]	Provide an adequate supply of sanitary products to the refugee camps	HIGH
Policy professionals	UNHCR [4]	Prevent gender-based violence, provide health care and sanitation, supporting education and skills development	MEDIUM
NGO	Islamic Relief (International Aid Agency) [5]	Help access sanitation facilities (latrines and bathing cubicles) and educate women by debunking common misconceptions related to menstruation	MEDIUM
Other actors	Bangladesh Government	Stop classifying education institutions within the refugee camp as illegal and allow menstruation education for women and men [11].	HIGH

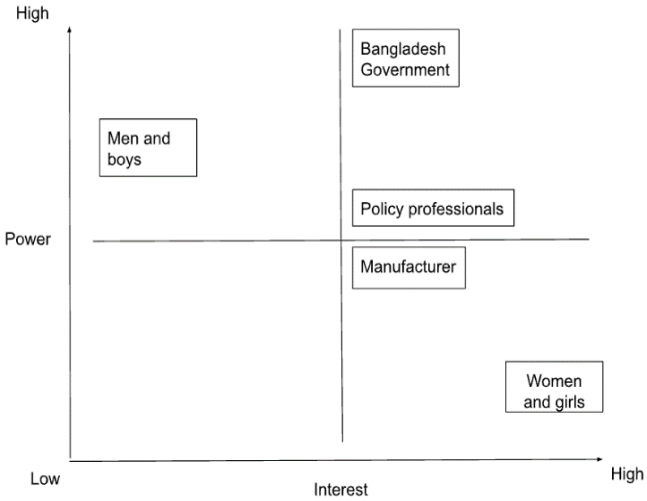

Figure 1: Diagram showing the stakeholders mapping

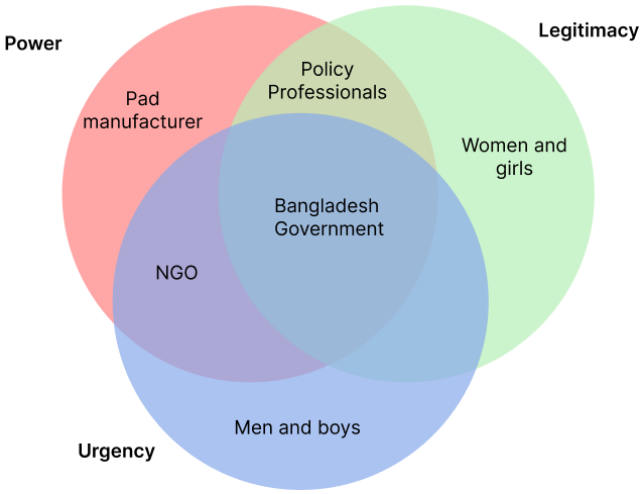

Figure 2: Stakeholder Salience model

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Objectives & Criteria

Table 2 summarizing the objectives and the criteria they meet

Objective	Criteria
Spread awareness on menstrual hygiene	• Increase the educational level of female adolescents from 17% to 50% [6]. • Increase premenstrual knowledge of girls aged 8-14 to 70% [3], [7]
Improve access to menstrual products	• Increase the supply of clean and safe pads from 20.79% to 70% to menstruating females [3]. • Supply the 50% of women who use cloths with menstrual products. • Develop a unified MHM approach and collaborate with nearby health facilities, NGOs, government offices, and schools [9].
Enhance sanitation infrastructure	• Develop efficient menstrual hygiene management MHM system to treat up to 90% of menstrual waste [8].

Risks and Constraints

Risks

1. Harassment and Violence
   - Women attempting to receive education or gain access to menstrual hygiene resources may face harassment, threats, and violence. [12]
2. Social Stigma and cultural resistance
   - Efforts to educate about menstrual hygiene are stigmatized, resulting in social ostracism for the participants. Deeply ingrained cultural religious beliefs may lead to strong resistance from community members who see this as an attack on their values.
3. Security Risks
   - In politically unsafe environments [10], this leads to increased security risks for educators and participants.

Constraints

1. Resource and infrastructure Limitations
   - Limited access to resources such as education materials and venues, sanitary products.
   - Poor infrastructure [11] (inadequate sanitation facilities, lack of private space for girls) makes it difficult to implement effective menstrual hygiene practices.
2. Logistical Issues and financial constraints
   - Hard to coordinate education campaigns within a densely populated camp.
   - A large amount of funds required to set up adequate sanitation facilities.
3. Cultural and religious constraints
   - Opposition and resistance to change from deeply religious populace

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